

Customer & MVP Blueprint

01 EXECUTIVE SUMMARY

LifeLink is a real-time emergency healthcare coordination platform that bridges the critical gap between patients requiring blood, platelets, or organ support and verified nearby donors and blood banks. Every minute of delay in emergency blood sourcing increases mortality risk. India alone faces a shortfall of ~3 million blood units annually. LifeLink's AI-powered matching engine, smart alerts, and trusted donor network aim to cut emergency resource procurement time from hours to under 15 minutes, directly improving survival outcomes during medical crises.

02 IDEAL CUSTOMER PROFILE & BUYER PERSONA

SEGMENT	PROFILE	SIZE / PRIORITY
Hospitals & Trauma Centers	Tier-1 & Tier-2 hospitals with blood banks; high ER throughput; frustrated by manual blood sourcing	★★★ Primary
Blood Banks & NGOs	Licensed blood banks needing better donor outreach; NGOs running donation drives	★★★ Primary
Verified Donors	18–55 yrs, health-conscious, app-savvy, motivated by altruism or community recognition	★★■ Supply-side
Patients & Families	Emergency seekers, often low-digital-literacy; rely on hospitals or family networks to find donors	★★■ End-users
Corporate / CSR Partners	Companies running employee blood donation programs; seeking measurable social impact metrics	★■■ Expansion

BUYER PERSONA	Dr. Priya Nair — Head of Blood Bank, Multi-specialty Hospital
Age / Role	38 Blood Bank In-charge, 400-bed hospital, Hyderabad
Goals	Zero delay in emergency blood fulfillment; accurate inventory; regulatory compliance
Pain Points	Manual calls to 10+ donors per emergency; outdated contact lists; no real-time donor status
Buying Power	Recommends tech vendors to hospital admin; budget ~\$2–5L/yr for operational tools
Tech Comfort	High — uses EHR systems daily; prefers mobile-first dashboards
Trigger	Lost a patient due to O-negative blood sourcing delay → proactively seeks solution

03 TOP 10 CUSTOMER PAIN POINTS

#	PAIN POINT	WHO FEELS IT	SEVERITY
1	Manual, phone-based donor search wastes critical minutes in emergencies	Hospitals	■ Critical
2	No real-time visibility into blood type availability across nearby banks	Hospitals, Patients	■ Critical
3	Donor databases are stale — contact info outdated, eligibility unknown	Blood Banks	■ High
4	Families resort to social media appeals, causing panic and misinformation	Patients & Families	■ High
5	Geographic mismatch — donors and need not co-located efficiently	All stakeholders	■ High
6	No notification system for rare blood groups (AB-, O-)	Blood Banks	■ High
7	Donor fatigue — lack of recognition/incentives leads to drop-off	Donors	■ Medium
8	Regulatory friction in cross-hospital blood transfer requests	Hospitals	■ Medium
9	No unified platform — blood, platelets, organ needs fragmented	Hospitals	■ Medium
10	Lack of post-donation follow-up erodes trust and re-donation rates	Donors	■ Low-Med

04 CUSTOMER JOURNEY

STAGE	AWARENESS	CONSIDERATION	PURCHASE / ONBOARD	RETENTION
Trigger	Blood emergency; news about shortage; doctor recommendation	Searching for digital solutions vs manual processes	Hospital admin signs MoU; blood bank integrates API	Recurring emergencies handled faster; ROI proven

Actions	Googles 'blood donor app India'; hears from peer hospital	Demos LifeLink; compares with spreadsheet workarounds	Signs up; registers donor pool; trains staff	Reviews monthly reports; expands to more departments
Emotions	■ Panic, urgency	■ Skeptical, hopeful	■ Cautious optimism	■ Relief, trust built
LifeLink Role	SEO, case studies, hospital webinars, WhatsApp campaigns	Free pilot, live demo with real data, ROI calculator	White-glove onboarding, staff training, helpline	Monthly impact dashboards, gamified donor badges

05 KEY OBJECTIONS & BUYING TRIGGERS

OBJECTION	REBUTTAL	BUYING TRIGGER
Data privacy of patient & donor info	HIPAA/DISHA-compliant architecture; on-prem option	■ Patient death attributed to blood sourcing delay
Trust in donor verification accuracy	Govt ID + health screening integration	■ Regulatory audit flagging manual donor records
System reliability during crisis peak loads	99.9% SLA; offline fallback mode	■ Peer hospital publicly reduces response time with tech
Integration with existing HIS/EHR systems	REST API + HL7 FHIR support; free integration support	■ High-profile case amplified on social media
Staff adoption & change management	WhatsApp-native interface; minimal training needed	■ Government mandate for digital blood bank records
Cost justification for smaller hospitals	Per-request pricing model; no upfront capex	■ CSR blood drive failure due to poor donor turnout

06 MVP RECOMMENDATION

BUILD FIRST ■	DO NOT BUILD YET ■	SUCCESS METRICS ■
1. Donor Registry — verified profiles with blood type, location, last donation 2. Smart Alert Engine — SMS/WhatsApp push to compatible nearby donors 3. Hospital Dashboard — real-time blood request creation & status tracking 4. Compatibility Matcher — ABO/Rh matching algorithm 5. Request Fulfillment Flow — accept → travel → confirm loop	Organ matching — regulatory complexity, separate framework needed Payments/rewards — adds compliance burden at MVP stage Consumer B2C app — focus B2B hospitals first for trust & scale AI demand forecasting — needs data; build after 6 months Multi-language UI — English + 1 regional language is enough	Primary: Time-to-fulfillment < 15 min (target vs 2–4 hr baseline) Activation: 5 hospitals live within 30 days Supply: 500 verified donors onboarded in pilot city Quality: >80% alert response rate within 30 min Retention: Hospital re-use rate > 3 requests/month

07 MoSCoW PRIORITIZATION

MUST HAVE ■	SHOULD HAVE ■	COULD HAVE ■	WON'T HAVE ■
- Donor registration & verification - Blood type compatibility engine - Real-time SOS alert (SMS/WA) - Hospital request dashboard - Donor availability toggle - Basic admin panel	- Location-based radius filter - Donor history & certificates - Blood bank inventory sync - In-app notifications - Basic analytics dashboard - Multi-hospital support	- Gamification & badges - CSR corporate portal - Platelet-specific workflows - Auto-escalation rules - API for HIS integration - Multilingual support	- Organ matching module - In-app payments - AI demand forecasting - Telemedicine features - Insurance tie-ups - Consumer B2C app v1

08 PRICING HYPOTHESIS & TOP 5 RISKS

TIER	TARGET	MODEL	PRICE
Starter	Small clinics/NGOs	Free (donor network)	■0
Hospital	50–500 bed hospitals	Monthly SaaS	■8–15K/mo
Enterprise	Hospital chains	Annual license + API	■1.5–5L/yr
Pay-per-use	Occasional users	Per successful match	■500/request
CSR Pack	Corporates	Drive management	■25K/drive

#	RISK	MITIGATION
1	Donor supply < demand at launch	Partner with existing NGOs; seed with 200+ donors pre-launch
2	Hospital inertia — slow procurement cycles	Start with 2–3 champion hospitals; get case studies fast
3	Regulatory compliance (CDSCO, state blood policy)	Engage healthcare lawyer; position as coordination layer, not bank
4	Trust deficit — wrong match or failed delivery	Human verification step before fulfillment; SLA guarantees
5	Well-funded competitor (eRaktKosh, NGOs scaling)	Hyper-local depth + real-time UX advantage; focus Tier-2 cities

09 30-DAY MVP PLAN

WEEK	FOCUS	KEY DELIVERABLES	OWNER
Week 1 Days 1–7	Discovery & Setup	• Interview 10 hospital blood bank heads • Define donor registration flow & data model • Set up dev environment, repo, CI/CD • Draft data privacy policy (DPDP Act)	Founders + Tech Lead
Week 2 Days 8–14	Core Build	• Build donor registry & onboarding (web + WhatsApp) • Implement blood type compatibility algorithm • Create hospital SOS request form • Build SMS/WA alert dispatch engine	Tech Team
Week 3 Days 15–21	Pilot Prep	• Onboard 200 verified donors in pilot city • Sign MoU with 2 partner hospitals • Internal QA + stress testing of alert system • Train hospital staff on dashboard (30 min session)	Ops + Sales
Week 4 Days 22–30	Launch & Learn	• Go live with pilot hospitals • Handle first 10 real emergency requests • Collect NPS from hospital coordinators & donors • Weekly metrics review; rapid iteration sprint	All Hands

10 FOUNDER ACTION SHEET — TOP 10 NEXT ACTIONS

WHEN	#	ACTION
TODAY	1	Call 3 hospital blood bank heads — ask: 'How do you find donors in an emergency today?'
TODAY	2	Map 5 existing blood donor NGOs in your target city for potential supply-side partnerships
DAY 3	3	Set up a WhatsApp group with 20 potential donors — test message response time manually
DAY 5	4	Draft a 1-page 'LifeLink Pilot Agreement' — offer 90-day free pilot to first 3 hospitals
DAY 7	5	Register the entity; consult a healthcare lawyer on blood coordination compliance
DAY 10	6	Build a Typeform/no-code donor registration flow + Google Sheets backend — no-code MVP first
DAY 14	7	Run a simulated emergency drill with 1 hospital partner to measure baseline response time
DAY 18	8	Post 3 LinkedIn case studies: 'What happens when a hospital can't find O- blood in 2 hours?'
DAY 21	9	Apply to 1 healthtech accelerator (e.g., Villgro, Nasscom CoE) for funding & credibility
DAY 28	10	Review Week 1–3 data; decide: build native app OR double down on WhatsApp-native experience

11 SCORES & FINAL VERDICT

Customer Clarity

82



Hospitals & blood banks are well-defined; donor segment needs sharper targeting

Problem Severity

91



Life-or-death urgency; massive unmet need with quantifiable mortality impact

PMF Potential

78



Strong signal — analogous global platforms (KoroConnect, Bloodlife) show traction

MVP Readiness

74



Core tech is achievable in 30 days; supply-side bootstrapping is the real challenge

ONG DEMAND SIGNAL

LifeLink addresses a high-severity, life-critical problem with a clear and reachable market. The problem is validated, the market is large, and the MVP is achievable within 30 days. Key risks are mitigated by scaling hospital demand. Prioritize 1 city, 3 hospitals, 500 donors, and 1